No. 24-1869

IN THE UNITED STATES COURT OF APPEALS
FOR THE NINTH CIRCUIT

AILA CURTIS, *et al*,

Plaintiffs-Appellants,

v.

GOVERNOR JAY INSLEE, *et al*

Defendants-Appellees.

On Appeal from the United States District Court
for the Eastern District of Washington
No. 3:23-cv-5741-RJB
Hon. Robert J. Bryan

APPELLANTS' OPENING BRIEF

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I. INTRODUCTION

The U.S. Congress, not governors or hospital CEOs, determines the conditions under which an investigational new drug is introduced into commerce. The U.S. Congress, not Appellees, determines when a national emergency will be declared and how it will administer its resources during the emergency for the public benefit. In 2020, the federal government declared a national emergency, purchased all COVID-19 drugs, labeled those drugs as investigational, immunized them from liability under the PREP Act, and then established the CDC COVID-19 Vaccination Program whereby states that volunteered to participate under their prerogative would administer the drugs to the public under very strict guidelines designed by Congress to ensure the constitutional and legal rights of the American people were protected at all times during the declared emergency. (3-ER-425, 441, 443, 464, 542) Despite Congress establishing a legal mechanism to introduce unapproved drugs into commerce during a declared emergency, it mandated in unambiguous language that no person has the authority to require another person to use the investigational drugs. (3-ER-445-446) Despite the U.S. Congress establishing a mechanism to shield persons from liability under the PREP Act, it mandated that no person could require another person to use an authorized product or participate in an activity under the Act. (2-ER-65; 3-ER-374)

The federal government could not enact legislation, issue an executive order, or establish a regulation requiring a person to inject unlicensed investigational drugs into their bodies as a condition of anything, as such an act would violate a person's fundamental right to refuse investigational medical treatment without penalty or pressure. (3-ER-348-349) Therefore, the federal government established the CDC COVID-19 Vaccination Program to delegate to states the constitutional and legal obligations it owed to all persons under its jurisdictional authority. Governor Inslee, under the authority of his office and the state's prerogative, willfully volunteered the State of Washington to perform on behalf of the federal government under the CDC Program. (3-ER-446-447) Just like the federal government, neither the State of Washington nor Governor Inslee could establish a legal requirement that a person must inject unlicensed investigational drugs into their bodies as a condition of anything. The State of Washington incorporated the CDC COVID-19 Vaccination Program as official state policy, requiring private parties, such as PeaceHealth, to sign the CDC Provider Agreement, ensuring that they, too, would perform on behalf of the state and federal government and protect the constitutional and federal statutory rights of potential recipients when administering the federal program. (3-ER-343-344)

The U.S. Congress was adamant that no person could be forced, under threat of penalty, to use authorized products or activities that are authorized under the

PREP Act or EUA Statutes, so it completely preempted states and the states' recruited parties from establishing, enforcing, or continuing in effect with any law or legal requirement that would deprive a person of their right to accept or refuse EUA/PREP Act investigational drugs. (3-ER-345, 404, 448, 449, 470)

Without exception, Appellees had one primary ministerial duty to perform under the Fourteenth Amendment, federal laws, state laws, and federal agreements regulating the emergency protocols, which was to accept Appellants' freely chosen option of whether to accept or reject EUA/PREP Act investigational drugs. (2-ER-65) Without question, Appellees violated their federal agreements, state agreements, Fourteenth Amendment obligations, and ministerial duties when establishing a "vaccination" requirement exclusively relying on the federally owned COVID-19 investigational drugs, placing Appellants under threat of penalty if they chose not to be injected with the federal drugs and ultimately enforcing the *ultra vires* requirement completely prohibiting Appellees from continuing with their careers within the state's healthcare industry to the detriment of the state's healthcare system during a pandemic. (3-ER-440, Inslee Proclamation) (3-ER-460, PeaceHealth Policy).

What is Appellees defense? Because a pandemic ensued, they could ignore the emergency protocols they volunteered to perform, which were designed to protect Appellants' fundamental rights during a federally declared emergency. (2-

ER-195, PeaceHealth) (3-ER-315, Inslee) Instead, Appellees established their own protocols, outside the scope of their authority, as they saw fit, not how the Republic, under the watchful eyes of the Fourteenth Amendment, mandated. Now that they face liability for their atrocious behavior, they have concocted a scheme to claim that the EUA/PREP Act "Pfizer-BioNTech COVID-19 Vaccine" was not investigational (2-ER-199-200) (3-ER-324-325, Inslee) but actually approved because the FDA approved COMIRNATY®, a legally distinct drug. However, as discussed below, not only does such a claim conflict with federal law and judicially noticeable documents issued by the FDA, it implies that the HHS Secretary and Pfizer committed fraud by continuing to manufacture, distribute, and administer the EUA drug when it had an FDA-licensed drug available within the marketplace. This implication was improperly upheld by the district court.

II. JURISDICTIONAL STATEMENT

The United States District Court for the Western District of Washington, District Judge Robert Bryan, granted a motion to dismiss the complaint and rendered judgment against the Appellants. (1-ER-2.) The judgment adjudicated all federal claims as to all parties. (1-ER-2.) The District Court's jurisdiction was established under 28 U.S.C. § 1331 as a civil proceeding arising under the laws of the United States, specifically 42 U.S.C. § 1983 (3-ER-427.)

Under 28 U.S.C. § 1291 the Court of Appeals has jurisdiction from all final decisions of the federal district courts. The Appellants filed a timely Notice of Appeal on March 26, 2024. (3-ER-563.)

III. ISSUES PRESENTED

I. Whether the District Court erred when it failed to accept Appellants' factual allegations as true that the only drugs available to them to comply with Defendants' vaccination requirements were EUA/PREP Act investigational drugs wholly owned by the federal government and not licensed for any indication.

II. Whether the District Court erred when it failed to address Appellants' allegation that the Supremacy Clause and the express preemption language in the PREP Act preempted Appellees' challenged conduct.

III. Whether the District Court erred when it granted Governor Inslee's motion to dismiss and that he was entitled to qualified immunity.

IV. Whether the District Court erred when granting PeaceHealth's Motion to Dismiss and failing to find that it was acting under color of law.

V. Whether the District Court erred when it found that Defendants could penalize Appellants for exercising their right to refuse EUA/PREP Act investigational drugs.

VI. Whether the District Court erred when it denied Plaintiffs-Appellants' Motion for Leave to File Second Amended Complaint. (2-ER-77)

IV. STATEMENT OF THE CASE

Appellants filed suit against Gov. Inslee and PeaceHealth after they penalized Appellants for exercising their Constitutional and federal statutory rights to refuse unwanted investigational drugs that were wholly owned by the federal government and offered only as an Emergency Use Authorization drug, under a declared emergency, through the CDC COVID-19 Vaccination Program and immunized under the PREP Act from liability. (3-ER-424-495) The Defendants filed Motions to Dismiss. (2-ER-192; 3-ER-312). The lower court improperly granted the Motions to Dismiss and entered Judgment on behalf of Defendants-Appellees. (1-ER-2, 3, 24, 45). Appellants filed a Rule 59(E) Motion to Alter or Amend Ruling, or in the alternative, Motion for Leave to File Second Amended Complaint, which the district court denied. (2-ER-77; 1-ER-3) Appellants timely filed a Notice of Appeal. (3-ER-563).

V. TERMINOLOGY

Throughout the underlying pleadings, Appellees failed to use precise language regarding the formal names of Pfizer's COVID-19 vaccine technology. The district court followed that imprecision and mischaracterized Appellants' allegations throughout the ruling when asserting that Appellants claimed that the "Pfizer-BioNTech vaccine was not fully approved in August of 2021" by conflating the generic phrase "Pfizer-BioNTech COVID-19 vaccine," lower case "v" (i.e.,

Pfizer's COVID-19 vaccine technology in general) with "Pfizer-BioNTech COVID-19 Vaccine" uppercase "V" (i.e., Pfizer's formal name for its investigational new drug under application 19736).[1] (1-ER-48). Therefore, when the district court stated that the Appellants claimed that "Pfizer-BioNTech vaccine was not fully approved in August of 2021," it was literally saying that the Appellants claimed that Pfizer's COVID-19 vaccine technology was not fully approved, which is in error, as noted in Appellants stating that Comirnaty was approved. (3-ER-356) (3-ER-353, "E")

Appellants, however, are specific in the use of terminology, specifically differentiating between Pfizer-BioNTech COVID-19 vaccine, Pfizer-BioNTech COVID-19 Vaccine, and COMIRNATY®, the name of Pfizer's only product approved by the FDA for general commercial marketing. (2-ER-251)

Therefore, the district court erred by not differentiating between "COMIRNATY®," "Pfizer-BioNTech COVID-19 Vaccine," and "Pfizer-BioNTech COVID-19 vaccine." Appellants point this out because their entire complaint is based on the Pfizer-BioNTech COVID-19 Vaccine's EUA drug label and how the laws applicable to that label (e.g., the EUA Statute and the PREP Act) impact Appellants' constitutional and legal rights. Drug labels reflect which laws apply to that drug. Thus, not using the formal drug labeling names only serves to confuse, not

[1] The court should be aware that the use of the capitalized word "Vaccine" in Pfizer's investigational drug label, "Pfizer-BioNTech COVID-19 Vaccine," does not indicate its FDA legal classification, only part of its formal investigational name.

enlighten. Moreover, Appellants spent a considerable amount of space discussing drug labeling laws, the legal distinctions between Pfizer's two drugs, and how those facts impact the Appellants' legal rights, which supports their claims for judicial relief. (2-ER-250-259). Simply because Pfizer's two drugs shared the same formulation on paper starting on August 23, 2021, is immaterial to the instant action because they do not share the same labeling, which means they do not share the same laws, and laws, not formulations, are Appellants' causes of action.

VI. SUMMARY OF THE ARGUMENT

The district court's ruling should be reversed because it treated Appellants' legal claims involving EUA/PREP Act investigational drugs as if they were claims only involving FDA-licensed drugs. (3-ER-27) Thus, the ruling, as applied to each count, is grounded on a legal premise not argued by Appellants and conflicts with judicially noticeable documents, federal law, and federal agreements that are in the record.

Therefore, the district court's legal foundation relating to Appellants' allegations is flawed because it starts with the incorrect premise that the EUA/PREP Act Pfizer-BioNTech COVID-19 Vaccine was somehow transformed into an FDA-licensed drug on August 23, 2021, the day COMIRNATY® received its FDA-license for certain age groups. Although not necessary at this stage of the litigation because all factual allegations must be accepted as true, Appellants nevertheless

provided the court with indisputable proof that the FDA notified Appellees on August 23, 2021, that the FDA issued a marketing license to COMIRNATY®, but also re-issued the EUA to Pfizer-BioNTech COVID-19 Vaccine. (2-ER-276-288) A drug cannot be issued an EUA if it is FDA-licensed and available for its intended emergency use (21 U.S.C. § 360bbb-3(a)(2)(A),(c)(3)). So, the district court's improper application of the FDCA to "Pfizer-BioNTech COVID-19 Vaccine" is a reversible error.

The FDA was unambiguous that despite approving COMIRNATY®, it had not approved "Pfizer-BioNTech COVID-19 Vaccine" but rather classified it as an investigational new drug under application 19736. (2-ER-284, 286) Moreover, the FDA placed a duty upon all parties involved in the drug's legal and physical administration under the state-run CDC COVID-19 Vaccination Program mandating that all descriptive printed matter, advertising, and promotional material relating to the use of "Pfizer-BioNTech COVID-19 Vaccine" clearly and conspicuously shall state that: "This product **has not been approved or licensed** by FDA, but has been authorized for emergency use by FDA, under an EUA." (Emphasis added) (2-ER-286)

Appellants were clear in their opposition to Gov. Inslee's Motion to Dismiss regarding the true state of availability of COMIRNATY®:

> Defendant continues to mislead the court by claiming that "Plaintiffs could have complied with the Proclamation by

> obtaining the fully authorized Pfizer vaccine, which is not an 'emergency use' product subject to the statute. 21 U.S.C. § 360bbb-3(e)(1)(A)." (Doc 18, p. 21). The statement that Plaintiffs could access licensed drugs is patently false, and Defendant has cited to no evidence to prove otherwise. Not only is this court required to accept as true Plaintiffs' factual allegations regarding the non-availability of Comirnaty, but the FDA and Pfizer informed the public that although Comirnaty was licensed for commercial use, the drug was never available in the U.S. Moreover, the FDA has issued numerous EUAs for "Pfizer-BioNTech COVID-19 Vaccine" since August 23, 2021. The FDA is prohibited from issuing an EUA if a licensed product exists to address the emergency condition, so by claiming that Plaintiffs could have used Comirnaty, Defendant is essentially stating that the HHS Secretary has violated federal law since August of 2021 by issuing additional EUAs for "Pfizer-BioNTech COVID-19 Vaccine." (3-ER-356-357)

Instead of relying on the FDA documents submitted by Appellants, the district court arrived at its conclusion by taking judicial notice of a "public notice" issued by the CDC stating:

> Pfizer-BioNTech (COMIRNATY®) received U.S. Food and Drug Administration (FDA) approval on August 23, 2021, for individuals 16 years of age and older. Once vaccines are approved by the FDA, companies can market the vaccines under brand names. COMIRNATY® is the brand name for the Pfizer-BioNTech COVID-19 Vaccine. Now that the FDA-authorized Pfizer-BioNTech COVID-19 vaccine has been approved by the FDA for individuals 16 years of age and older, it will be marketed as COMIRNATY®. (1-ER-48)

Appellants did not and do not dispute that the FDA approved COMIRNATY® for general commercial marketing pursuant to 21 U.S.C. § 355. (2-ER-254) However, the CDC only informed the public that COMIRNATY® was approved

and that sometime in the future Pfizer would market COMIRNATY® to the public, as noted in the "will be marketed" statement.

The FDA referenced both drugs stating, "The Fact Sheet for Recipients and Caregivers was updated as the Vaccine Information Fact Sheet for Recipients and Caregivers, which comprises the Fact Sheet for the authorized **Pfizer-BioNTech COVID-19 Vaccine and** information about the FDA-licensed vaccine, **COMIRNATY** (COVID- 19 Vaccine, mRNA)," (2-ER-277) (emphasis added) So, while the FDA distinguishes between the legal classification of the two drugs, the district court did not, which is reversible error.

Therefore, the Appellants not only alleged that the FDA classified "Pfizer-BioNTech COVID-19 Vaccine" as investigational, but Appellants provided the court with a judicially noticeable document whose sources of authority cannot be reasonably questioned. The FDA, not the CDC, determines a drug's classification. (21 U.S.C. § 355(a)) Moreover, the CDC only referenced in its "public notice" the future marketing plans of COMIRNATY® without speaking to the current legal status of "Pfizer-BioNTech COVID-19 Vaccine," with which the CDC was intimately familiar since it managed the CDC COVID-19 Vaccination Program that exclusively relied on investigational drugs.[2]

[2] Appellants' classification arguments apply equally to COVID-19 drugs manufactured by Moderna and J&J.

Appellants informed the court that an EUA cannot be issued if a "licensed product exists to address the emergency condition," citing 21 U.S.C. § 360bbb-3(c)(3) (3-ER-357) and that the HHS Secretary continued issuing EUAs for "Pfizer-BioNTech COVID-19 Vaccine" starting on and after August 23, 2021. (3-ER-356) Therefore, it is curious why the district court stated:

> "The Plaintiffs premise their argument and much of their case on the notion that the only vaccines available at the time of the Proclamation were 'experimental' contending that the Pfizer-BioNTech, Moderna, and Johnson & Johnson vaccines were only authorized pursuant to 21 U.S.C. § 360bbb-3, the EAU [sic] statute." (1-ER-59)

Appellees did not produce a single document, press release, or notification from any COVID-19 drug manufacturer or federal agency that COMIRNATY® or any other FDA-licensed COVID-19 drug had actually been shipped or made available to Appellants at any material time. Moreover, Appellants provided the court with judicially noticeable documents informing the court that Pfizer informed the CDC that "Pfizer received initial FDA BLA license on 08/23/2021 for its COVID-19 vaccine for use in individuals 16 and older (COMIRNATY). At that time, the FDA published a BLA package insert that included the approved new COVID-19 vaccine trademark COMIRNATY and listed 2 new NDCs (0069-1000-03, 0069- 1000-02) and images of labels with the new tradename. **These NDCs will not be manufactured**." (Emphasis added) (2-ER-104-105) The NDCs are the FDA-approved marketing codes that must be attached to all boxes, labeling, and drug vials

labeled as COMIRNATY. Pfizer informed the CDC that it had no intention of manufacturing under those NDCs, which it did not, and it is why the HHS Secretary issued EUAs for the investigational drugs,[3] (3-ER-357) which, according to the FDA and Pfizer, were only comprised of six-dose vials manufactured before August 23, 2021, in non-BLA-approved facilities. Thus, laws, case law, regulations, and other legal requirements applicable only to licensed drugs involving vaccine mandates never came into being for all times material.

Rather than addressing Appellants' contention that only EUA/PREP Act investigational drugs were available for compliance with the mandates, (3-ER-441, 460) the district court adopted the reasoning of an Oregon District Court in *Johnson v. Brown*, 567 F.Supp.3d 1230 (D. Or. 2021), which took judicial notice of a "press release" from the FDA and focused on the *medical* interchangeability of the products but did not discuss the different laws applicable to the different labels. Moreover, the parties before the *Johnson* court did not present the allegations that Appellants make herein regarding their right to a voluntary PREP Act program, the duties owed by Appellees under the CDC COVID-19 Vaccination Program, and the federal

[3] The HHS Secretary issued EUAs for ages 16 and older for Pfizer-BioNTech COVID-19 Vaccine on: August 23, 2021), (September 22, 2021), (October 20, 2021), (November 19, 2021), (December 9, 2021), (December 16, 2021), (January 3, 2022), (March 29, 2022), (May 17, 2022), (June 17, 2022), (July 8, 2022), (August 31, 2022), (October 12, 2022), (December 8, 2022), (March 14, 2023), (April 18, 2023), and on (April 28, 2023)

preemption issues Appellants raise herein. (1-ER-48) So, the *Johnson* court does not help to resolve the legal issues involved in the Motions to Dismiss in the instant case.

Appellants maintain that laws applicable to a drug's labeling (i.e., the EUA Statute and the PREP Act) are the laws that establish the relevant parameters regarding the use of those drugs and its why the FDA specifically informed Appellees of their duties relating to the product's "labeling" throughout each of its EUAs. (3-ER-399,409,543,544,546,547) A label is the only means to determine the regulatory scheme the drug is governed by and the label regulates the $600b pharmaceutical industry. (2-ER-106) The district court erred by not addressing those laws and, instead, holding that the drugs were something other than investigational without informing anyone of what it believed to be the drug's legal classification and its applicable laws.

Appellants presented the court with a declaration by Peter Marks, Director of the Center for Biologics Evaluation and Research (CBER), United States Food and Drug Administration (FDA), responsible for vaccine regulation, informing the court that the FDA included the use of the word "interchangeable" only to mean *medical* to avoid "unnecessary operational complications" but there were "*legal* distinctions" as noted in the product's labeling. (2-ER-289-293)(emphasis added).

The district court stated:

> "He [Peter Marks] points out that COMIRNATY® is labeled in one manner and the Pfizer-BioNTech vaccine is labeled

> differently. Dkt. 36-2 at 5. That the labels are different is a fact that 'can be accurately and readily determined from sources whose accuracy cannot be reasonably questioned' and so is judicially noticed here. However, a difference in labeling does not change the outcome of this case." (1-ER-30-31).

The district court does not give drug labeling laws as established by Congress their legal effect nor accept as true Appellants' arguments as to how the laws applicable to those drug labels impact their legal right to refuse without penalty or pressure for the sole reason that the FDA approved a legally distinct drug sharing formulation on paper with COMIRNATY as of August 23, 2021. (2-ER-254-255, 277 (FN8); 3-ER-456-457))

The district court's ruling regarding each of Appellants' causes of action ignored the fact that the drugs under Appellees' mandates were categorized by the FDA as investigational, only authorized for emergency use under 21 U.S.C. § 360bbb-3, immunized from liability under the PREP Act, funded by the DoD, owned by the federal government, and subject to the terms and conditions of the State's CDC COVID-19 Vaccination Program. Moreover, the district court erred by ruling in complete isolation of the alleged facts that the federal government, state government, and PeaceHealth owed Appellants constitutional obligations because of the drug's classification as assigned to its labeling and the emergency protocols under which the federal government released it into commerce. (3-ER-434-344, 348-349)

Therefore, the ruling, as applied to each cause of action, is in error because the district court ruled on those issues as if the drugs were operating under laws and programs not requiring constitutional and legal duties of the federal government, state government, and Appellees for Appellants' benefit, namely, laws that only apply to FDA-classified licensed drugs not introduced into commerce under emergency expanded access protocols.

Therefore, the drugs at issue were federally owned EUA/PREP Act investigational drugs offered exclusively through the CDC COVID-19 Vaccination Program, requiring Organizations (such as PeaceHealth) to sign the CDC Provider Agreement and follow numerous specific program requirements under the state's exclusive authority and complete control, The CDC COVID-19 Vaccination Program and applicable laws granted Appellants the following rights:

(1) Option to accept or refuse under 21 U.S.C. § 360bbb-3(e)(1)(A)(ii)(III), (3-ER-498, 12(a))

(2) Right to have the option to refuse to be treated equally with the option to accept in accordance with the Fourteenth Amendment, (3-ER-450)

(3) Right to be educated about the voluntary nature of a PREP Act program (42 U.S.C. § 247d-6e) (2-ER-124)(3-ER-374, #7)

(4) Right to give legally effective informed consent under 45 C.F.R. § 46.116, (3-ER-349)

(5) Right to give informed consent under 10 U.S.C. § 980 since the subject drugs were DoD-funded investigational drugs, (3-ER-370)

(6) Fundamental right to refuse an investigational drug under the Fourteenth Amendment's substantive due process provisions, (see, *infra)*

(7) Fundamental right to refuse unwanted investigational medical treatment, (see, *infra*)

(8) Right to give informed consent under Appellees' FWAs, (3-ER-453)

(9) State common law right to exercise a legal right without loss of employment, (3-ER-489, FN65)

(10) Preemption of state laws and legal requirements that conflict with Appellants' right to be educated about the voluntary nature of a PREP Act program. (2-ER-124)

(11) Preemption of state laws and legal requirements that conflict with Appellants' right to refuse an EUA drug without penalty or pressure. (3-ER-404)

(12) Fundamental right not to surrender Fourteenth Amendment due process protections by not volunteering to participate in a PREP Act countermeasure or activity. (3-ER-456)

The district court referenced case law only applicable to licensed drugs and nothing to provide jurisprudence that a person, acting on behalf of the federal and state government, can subject another person to federally owned investigational drug use under threat of penalty. It dismissed all claims brought by Appellants for the sole reason that it disputes the FDA's investigational classification of "Pfizer-BioNTech

COVID-19 Vaccine," thereby converting Appellants' causes of action under the emergency protocols involving only investigational drugs into causes of action involving only FDA-licensed drugs not under the emergency protocols. The district court erred by ruling on each count after ruling the drugs were not investigational or under an EUA and PREP Act immunity, which is the legal foundation that supports Appellants' claims.

VII. STANDARD OF REVIEW

This court reviews denials of Motions to Dismiss *de novo*.

VIII. ARGUMENT

A. The district court erred in holding that "Pfizer-BioNTech COVID-19 Vaccine" was not investigational.

Appellants presented the court with a judicially noticeable document from the FDA stating that Pfizer, Inc. had to submit to investigational new drug application 19736 for its "Pfizer-BioNTech COVID-19 Vaccine." (2-ER-284) Appellees were required to inform all persons that the drugs were not licensed or approved but under EUA. (2-ER-286-287) Appellants provided the court with the EUA and cited to the investigational classification of all drugs under Appellees' vaccination mandates as published in the Federal Register. (3-ER-442, FN4,5) Moreover, 21 U.S.C. § 360bbb-3(c)(3) dictates that no licensed drug can exist in the marketplace that has a legal indication to meet the intended emergency use as a condition of the EUA being

issued. Appellants alleged that the HHS Secretary continued issuing EUAs starting on August 23, 2021, and thereafter and can provide those EUAs at trial. (3-ER-357)

Appellees provided no judicially noticeable documents contradicting the FDA's official notice relating to the investigational classification of "Pfizer-BioNTech COVID-19 Vaccine." The district court's holding that Pfizer-BioNTech COVID-19 Vaccine was not investigational, not subject to the EUA Statute, nor offered under the CDC COVID-19 Vaccination Program as investigational use only is reversible error.

B. Preemption

The district court erred by not addressing Appellants' federal preemption allegations, but since this Court reviews the granting of a Motion to Dismiss *de novo*, Appellants address preemption here because it is a threshold issue related to Governor Inslee and PeaceHealth and was an issue ignored by the district court. (3-ER-Dkt. 8)

Federal preemption is rooted in the Constitution's Supremacy Clause, which provides that federal statutes "shall be the supreme Law of the Land." U.S. Const. art. VI, cl. 2. When state action conflicts with federal statutes, the Supreme Court stated that "[T]he question whether a certain state action is pre-empted by federal law is one of congressional intent." "The purpose of Congress is the ultimate touchstone" and "[t]o discern Congress' intent we examine the explicit statutory

language and the structure and purpose of the statute." (internal quotation marks and citation omitted) *Gade v. National Solid Wastes Management Assn.*, 505 U.S. 88 (1992).

The Supreme Court has long held that the intent of Congress is primarily discerned from the statute's text (*Medtronic, Inc. v. Lohr*, 518 U.S. 470, 486 (1996)), and the statutory construction is reviewed to ascertain how the federal law interacts with state law (*Id*). The Court also holds the doctrine of "presumption against preemption" "unless that was the clear and manifest purpose of Congress" (*Rice v. Sante Fe Elevator Corp.*, 331 U.S. 218, 230 (1947)) except where Congress expressly preempted the state action then the court "do[es] not invoke any presumption against pre-emption but instead focus[es] on the plain wording of the clause, which necessarily contains the best evidence of Congress' pre-emptive intent" (*Puerto Rico v. Franklin Cal. Tax-Free Trust*, 136 S. Ct. 1938, 1946 (2016)).

Appellants' specific preemption claims are that the Supremacy Clause and the express preemption language under the PREP Act preempt any state, and its recruited state actors, from establishing or continuing in effect with any law or legal requirement that conflicts with the option to accept or refuse participation in any product or activity authorized under the EUA Statute (21 U.S.C. § 360bbb-3) or the PREP Act (42 U.S.C. § 247d-6d). Appellants alleged that Appellees' challenged conduct conflicted with the legal requirements under the statutes related to

EUA/PREP Act investigational drugs (i.e., the option to accept or refuse), which, in fact, violated Appellants' Fourteenth Amendment and statutory entitlement rights. (3-ER-374, 404, 426, 448, 467, 480)

Appellants alleged in the court below that Congress preempted states from enforcing or continuing in effect with a state's at-will employment doctrine when that law is used solely for the purpose of interfering in Appellants' free consent under the statutes. Moreover, Appellants alleged that neither the state nor its recruited state actor, PeaceHealth, could establish a requirement for Appellants to seek a religious or medical exemption as the means to refuse use of EUA/PREP Act drugs (3-ER-469-470, 480) because such a requirement amends the HHS Secretary's conditions of authorization under each EUA and PREP Act Declaration which conduct is unambiguously preempted.

C. Emergency Expanded Access

21 U.S.C. § 360bbb-3 only provides the HHS Secretary authority during a declared emergency to exempt drugs from the normal method of being introduced into commerce under 21 U.S.C. § 355(a) by granting "expanded access" to drugs that are "not approved, licensed, or cleared for commercial distribution," and only under the condition that "there is no adequate, approved, and available alternative to the product for diagnosing, preventing, or treating such disease or condition" (21 U.S.C. § 360bbb-3(a)(2)(A); 21 U.S.C. § 360bbb-3(c)(3)). Once the product is

granted emergency "expanded access," the HHS Secretary is under a "required condition" to establish appropriate conditions that "healthcare professionals administering the product are informed…that the Secretary has authorized the emergency use of the product" and to establish separate "conditions designed to ensure that individuals to whom the product is administered are informed…of the option to accept or refuse administration of the product…." (21 U.S.C. § 360bbb-3(e) *et seq*.)

However, the intent of Congress that all persons use EUA/PREP Act investigational drugs only under voluntary conditions is clearly demonstrated in the statutory text by Congress denying the HHS Secretary "any authority to require any person to carry out any activity that becomes lawful pursuant to an authorization under this section" (21 U.S.C. § 360bbb-3(l)). (3-ER-445)

The EUA Statute establishes only the HHS Secretary's authority, and no state, political subdivision, or private party acting under color of law on behalf of the federal or state governments has the authority to add to or delete from the statute's requirements or prohibitions. (21 U.S.C. § 360bbb-3(a)(1)) The Supremacy Clause Doctrine dictates that because introducing unlicensed drugs into commerce under a federally declared national emergency is a field completely dominated by the federal government, no state or local government can establish or continue in effect with a law that conflicts or interferes with the right of individuals to freely choose one of

the two federally secured options under the EUA Statute (21 U.S.C. § 360bbb-3(e)(1)(A)(ii)(III)). Moreover, the equal protection provision of the Fourteenth Amendment mandates that state governments and persons acting on their behalf cannot treat the option to accept differently from the option to refuse and cannot apply a penalty to either option simply because the state and/or persons acting under color of law on its behalf disagrees with the individual's choice. (3-ER-350, 352, 469, 488)

The EUA statute's plain language mandates that a person must be made aware of their right to accept or refuse, which is a statutory entitlement or property interest.[4] Therefore, if Congress only authorizes the HHS Secretary to establish the conditions under which an unapproved drug or device can be introduced into commerce during a declared emergency and it prohibits the Secretary from having any authority to require a person to manufacture, distribute, administer, or use the product, then

[4]" To have a property interest in a benefit, a person clearly must have more than an abstract need or desire for it. He must have more than a unilateral expectation of it. He must, instead, have a legitimate claim of entitlement to it," and "Property interests, of course, are not created by the Constitution. Rather, they are created, and their dimensions are defined, by existing rules or understandings that stem from an independent source such as state law -- rules or understandings that secure certain benefits and that support claims of entitlement to those benefits. Thus, the welfare recipients in *Goldberg v. Kelly,* 397 U.S. 254 (1970), had a claim of entitlement to welfare payments that was grounded in the statute defining eligibility for them." *Board of Regents of State Colleges v. Roth*, 408 U.S. 564 (1972)

Appellees were at all times material also prohibited from establishing conditions that Congress expressly prohibits—nonconsensual use.

D. The PREP Act

42 USC 247d-6d(b)(8) (**PREP Act**) states that "During the effective period...no State or political subdivision of a State may establish, enforce, or continue in effect with respect to a covered countermeasure any provision of law or legal requirement that— (A) is different from, or is in conflict with, any requirement applicable under this section; and (B) relates to the...administration...of the covered countermeasure, or to any matter included in a requirement applicable to the covered countermeasure under this section or any other provision of this chapter, or under the Federal Food, Drug, and Cosmetic Act [21 U.S.C. 301 *et seq*.]." (emphasis added) (3-ER-345, FN5)

The U.S. Supreme Court held that when a statute "contains an express pre-emption clause," we "focus on the plain wording of the clause, which necessarily contains the best evidence of Congress'[s] pre-emptive intent." *Puerto Rico v. Franklin Cal. Tax-Free Trust*, 136 S. Ct. 1938, 1946 (2016) (quotation marks omitted).

The PREP Act requires a person to voluntarily surrender their right to bring common law causes of action for product liability, medical malpractice, fraud, and

battery; their rights to seek tort remedies for bodily harm caused by another member of society arising out of a covered countermeasure; their rights to be made whole for damages to their finances and emotional well-being which implicates a person's fundamental Fifth and Fourteenth Amendment Substantive Due Process rights. This demonstrates why Congress expressly preempts states or persons performing on behalf of the federal or state government under the statute from establishing, enforcing, or continuing in effect with a legal requirement that turns the "voluntary nature of the program"[5] into an involuntary one. Moreover, a person may not use the PREP Act as a procedural device to deprive a person of their due process rights.[6]

The PREP Act is unambiguous that the express preemption extends "to any matter included in a requirement applicable to the covered countermeasure…or under the Federal Food, Drug, and Cosmetic Act," which includes the "option to accept or refuse" under 21 U.S.C. § 360bbb-3(e)(1)(A)(ii)(III) (FDCA) (emphasis added). Appellees' "vaccination" mandates requiring involuntary use of covered countermeasures under the PREP Act conflicted with the voluntary nature of the program and Appellants' entitlement rights to accept or refuse under the FDCA. (2-ER-273) (3-ER-374, 479)

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[5] "The Secretary shall ensure...that potential participants are educated with respect to...the voluntary nature of the program…" (emphasis added) 42 U.S.C. § 247d-6e(c)

[6] *Perry v. Sindermann*, 408 U.S. 593 (1972) *quoting Speiser v. Randall*, 357 U.S. 513 (1958)

In effect, Appellees' "vaccination" mandates requiring the use of an EUA/PREP Act drug unlawfully amended the FDCA by establishing a prohibited act under 21 U.S.C. § 331, assigning a penalty to that prohibited act under 21 U.S.C. § 333, and voiding the option to refuse under 21 U.S.C. § 360bbb-3(e)(1)(A)(ii)(III). It also amended the conditions of authorization established by the exclusive authority of the HHS Secretary in each EUA and PREP Act declaration. (3-ER-442,540,542). These actions were also preempted by the well-established Supremacy Clause Doctrine[7] as it relates to federal legislation such as the FDCA and the PREP Act.

Appellants alleged that a state may not continue in effect with its at-will employment laws when a public or private person uses the law to penalize a person exercising their right to refuse EUA/PREP Act products. (2-ER-247) (3-ER-480).

When individuals consent to use an EUA/PREP Act product, they:

> (1) forfeit civil litigation rights to sue for resulting injuries, (3-ER-456-457)
>
> (2) allow their private identifiable and health information to be collected and shared with unknown persons and reasons, (*Id*)
>
> (3) become human subjects under research conditions where their use of the unapproved product will be monitored, studied, and cataloged by researchers, (*Id*)

[7] *Buckman Co. v. Plaintiffs' Legal Committee*, 531 U.S. 341 (2001); *Rice v. Santa Fe Elevator Corp.*, 331 U.S. 218, 230 (1947); *Arizona v. United States*, 567 U.S. 387, 399 (2012); *United States v. Locke*, 529 U.S. 89, 108 (2000).

> (4) and assume greater risks to their finances, safety, health, and legal rights. (*Id*)

With respect to a PREP Act covered countermeasure or EUA product, Congress expressly preempted all persons from establishing, enforcing, or continuing in effect any law or legal requirement that conflicts with the purely voluntary federal goals of the statutes because the loss of such fundamental rights can only result from Appellants voluntarily relinquishing those rights without penalty or pressure, not by Appellees' mandatory and coercive policies.

Therefore, the federal constitution prohibited Governor Inslee from establishing a legal requirement subjecting Appellants, under threat of penalty, to use EUA and PREP Act drugs by establishing a "vaccination" requirement relying only on those drugs for compliance. Because PeaceHealth was acting under color of law on behalf of the state and agreed to perform under each EUA, it too was restricted by the Supremacy Clause and the PREP Act's express preemption language from requiring Appellants' to use EUA/PREP Act investigational drugs. Moreover, the Supremacy Clause and PREP Act's express preemption language preempted PeaceHealth's use of the state's at-will employment laws as a "procedural

device"[8] to interfere in Appellants' right to give or withhold consent regarding the use of EUA/PREP Act investigational drugs.

E. The District Court erred when granting Gov. Inslee's Motion To Dismiss

Appellants alleged that Governor Inslee volunteered the State of Washington to perform for the federal government under the CDC COVID-19 Vaccination Program. Moreover, Appellants alleged that Governor Inslee's Proclamation exclusively relied on drugs only authorized for emergency use under the EUA Statute, offered exclusively through the CDC COVID-19 Vaccination Program, immunized from liability under the PREP Act, and FDA-labeled as investigational, and which invoked duties under the state's FWA. Moreover, Appellants alleged that since those facts are true, Governor Inslee could not subject Appellants, under threat of penalty, to the drug's use, and such requirements were preempted by the Supremacy Clause as it relates to the EUA Statute and express preemption language under the PREP Act. Appellants also alleged that Governor Inslee's enforcement of his mandate violated their fundamental Fourteenth Amendment rights to Equal Protection, Due Process, and Privacy and also their statutory rights to refuse products

[8] "In practical operation, therefore, this procedural device must necessarily produce a result which the State could not command directly. It can only result in a deterrence of speech which the Constitution makes free." *Speiser v. Randall*, 357 U.S. 513 (1958) The U.S. Congress makes rejecting unlicensed drugs a free right in which no government nor persons acting on behalf of the government can interfere nor establish a "procedural device" to exempt itself from its constitutional restraints.

and activities under the EUA Statute and PREP Act without penalty or pressure. Finally, Appellants alleged that Governor Inslee had ministerial duties under the CDC COVID-19 Vaccination Program, the State's FWA, EUA Statute, the PREP Act, and the Fourteenth Amendment involving the federally owned COVID-19 investigational drug that he willfully and recklessly violated leading to Appellants' claimed injuries. (3-ER-Dkt. 8)

The district court held that the drugs under Governor Inslee's Proclamation were not investigational and, as a result, not subject to the EUA Statute or to the terms and conditions of the CDC COVID-19 Vaccination Program relating to investigational drugs under an EUA. (1-ER-Dkt 32) The district court failed to address Appellants' PREP Act and preemption claims relating to Governor Inslee's Proclamation.

Curiously, the district court proceeded to rule on each count as if the question before it was how those counts related to FDA-licensed drugs, not investigational drugs under emergency access protocols or the PREP Act. Therefore, the district court's ruling on each count is unhelpful to this Court because it uses laws and cases neither alleged by Appellants nor material to the instant action because those laws and cases only apply to laws involving licensed drugs

Governor Inslee maintained that "Plaintiffs have failed to allege they were required to take any 'investigational' drug, their claim necessarily fails (3-ER-325)

and that 'Plaintiffs appear to suggest that 'Comirnaty'—the brand name assigned to the Pfizer vaccine after FDA approval—was somehow distinct from the Pfizer vaccine that had long been available to United States residents. *See, e.g.*, Opp'n at 15, 18-19. But that's untrue; the 'Pfizer BioNTech' vaccine that had long been available in the United States and "Comirnaty" are the same thing." (3-ER-413) However, Governor Inslee utilizes artful wording to confuse the court on the facts of the case by only stating that the Pfizer BioNTech vaccine technology (lowercase v) and COMIRNATY® are the same thing and not saying that the Pfizer-BioNTech COVID-19 Vaccine (uppercase V) and COMIRNATY® are the same thing because they are not the same thing legally nor do they share the same laws for their licensed indication.

Therefore, to present the idea that Appellants could have accessed a licensed drug, Gov. Inslee confuses the court by not referencing the formal drug label names in relation to his vaccine mandates. (3-ER-412-413). Moreover, because a pandemic ensued, he claims that his actions should be judged under a rational basis review despite Congress only authorizing the use of the EUA/PREP Act statutes *during a national emergency* and still they ensured all persons could not be subjected to involuntary participation. Governor Inslee did not dispute that he volunteered the State of Washington to perform under the CDC COVID-19 Vaccination Program, that he owed duties under the state's FWA, or that his conduct violated Appellants'

rights involving investigational drugs. He only contends that "Pfizer-BioNTech COVID-19 Vaccine" was not FDA-classified as investigational, thus implying it was not under legal requirements mandating he perform the ministerial duty of accepting a person's right to refuse, an assertion that cannot reconcile with the judicially noticeable positions of HHS, CDC, FDA, and Pfizer, Inc. on the subject.

F. The CDC COVID-19 Vaccination Program and Provider Agreement

The federal government's executive branch, owning the investigational drugs, owed all individuals the right to accept or refuse the EUA/PREP Act products without penalty or pressure (2-ER-106) because it was bound to comply with 45 C.F.R. Part 46, the FWA program, 10 U.S.C. § 980, PREP Act, and the Belmont Report, because the drugs were purchased using DoD and other public funding, under research conditions, labeled as investigational, and required participating individuals to prospectively surrender their constitutional rights to privacy and due process while also agreeing to assume more than minimal risk to their health, safety, and legal rights.

Therefore, it established the CDC COVID-19 Vaccination Program to ensure states that volunteered to administer the drugs on its behalf complied with the federal government's constitutional and legal obligations. The State of Washington has an FWA (3-ER-454) requiring it to comply with the same laws and regulations required of the federal government anytime a human is involved in a federally funded

investigational new drug and it is for this reason the CDC used a state's existing FWA to administer its program.

The CDC COVID-19 Vaccination Program established the CDC COVID-19 Vaccination Program **Provider Agreement** to ensure private parties the state recruited to perform on its behalf to help it achieve its goal of administering the federal investigational drug property also complied with the federal and state government's legal obligations. (3-ER-496-503)

The Provider Agreement was incorporated into official state policy when the State established its COVID-19 Vaccination Program, requiring medical facilities (e.g., PeaceHealth) to sign and comply with its terms and conditions. The State required all medical facilities not to sell any drugs or ancillary supplies relating to the drugs' administration (3-ER-497, #3), provide a drug Fact Sheet which required the medical facility to allow a person to access health counseling for free (*Id*, #4,5), "comply with all applicable requirements" under the EUA Statute, and any "requirements in any EUA that covers COVID-19 Vaccine" and "all applicable state and territorial vaccination laws." (*Id*, #12)

The Provider Agreement informed Governor Inslee that if the state and its medical facilities wanted to "receive one or more of **publicly funded** COVID-19 vaccines" "at no cost," they must "adhere to the" Program's requirements (emphasis added). (3-ER-497). The Provider Agreement could not be signed by a medical

facility without first going through and being approved by the State because the Program was "part of **collaboration** under the relevant state, local, or territorial immunization's cooperative agreement with CDC." (Emphasis added). (3-ER-443-444). The State owed the duty to perform the ministerial function of accepting Appellants' freely chosen option under any EUA because the drugs were classified as investigational. Moreover, the state was under no obligation to recruit any medical facility to administer the federal investigational drugs. It did so only under its exclusive prerogative. However, once it involved any private party to administer its property on behalf of the federal government, it was under a constitutional and contractual obligation to ensure its state actors would perform the ministerial duty of accepting the individual's free choice under the EUA Statute, PREP Act, and of their fundamental right to refuse an investigational drug and forfeiture of their Fourteenth Amendment due process and privacy rights. It could not delegate to the private party its required function of administering the federal drugs without delegating its constitutional and legal obligations. Therefore, the state required PeaceHealth and all other medical facilities to ensure potential recipients' consent rights through the Provider Agreement. The Organizations under the Provider Agreement did not have the legal authority to establish a policy requiring individuals to nonconsensually use the federal investigational drugs nor exempt themselves from the ministerial duty of accepting the Appellants' freely chosen option of whether to

accept or refuse. Moreover, neither Governor Inslee nor PeaceHealth could use employment policies to circumvent their duties because the legal duties owed by Appellees were to all individuals irrespective of Appellees' legal relationship to the individual. The state assured the federal government it would not require mandatory use of these drugs, and thus, no person the state recruited could require of Appellants that which the U.S. Congress expressly forbids—nonconsensual use.

Therefore, when Governor Inslee issued his Proclamation that exclusively relied upon the federal investigational drugs for compliance, he knowingly and willfully violated his voluntary agreement with the federal government under the CDC COVID-19 Vaccination Program and Provider Agreement. When PeaceHealth issued a vaccination policy requiring Appellants to inject the investigational drugs into their bodies outside of their consent, it knew it was violating its duties under the Provider Agreement and the EAU Statute. PeaceHealth has been administering investigational drugs under EUA for decades on an ongoing basis and knows it is prohibited from placing a person under threat of penalty for refusing an authorized product. (3-ER-476) That fact is why PeaceHealth claims the drugs were not investigational. In effect, Governor Inslee's Proclamation, as applied, required all medical facilities to commit fraud against the U.S. Government under the Provider Agreement by denying healthcare workers the right to refuse under the Program.

i. "Organizations"

Under the CDC COVID-19 Vaccination Program, the state recruited medical facilities to act on its behalf, requiring the executive staff (i.e., CEO, CMO, etc.) (3-ER-496) to sign the Provider Agreement as an "Organization." These "Organizations" agreed to be **collaborators** acting under color of law on behalf of the federal and state governments when administering the Program involving the "publicly funded" drugs.

The State and Organizations agreed to the fact that "all relevant officers, directors, employees, and agents of Organization involved in handling COVID-19 Vaccine understand and will comply with the agreement requirements listed above" and "the above requirements are material conditions of payment for COVID-19 Vaccine-Administration claims submitted by Organization to any federal healthcare benefit program…" and "non-compliance with the terms of Agreement may result in suspension or termination from the CDC COVID-19 Vaccination Program and criminal and civil penalties under federal law…"

Therefore, the "Organization," (i.e., PeaceHealth) as a whole, agreed to perform under the state-run program, not just the healthcare professionals it employed.

ii. "Any EUA" under the Provider Agreement

The state and Governor Inslee became the "Emergency Response Stakeholder" under each EUA (3-ER-542, FN 5), having duties including:

(1) to "identify vaccination sites to receive authorized Pfizer-BioNTech COVID-19 Vaccine **and ensure its** distribution and **administration**, consistent with the terms of this letter and CDC's COVID-19 Vaccination Program," (3-ER-546)(emphasis added)

(2) to "ensure that vaccination providers within their jurisdictions are aware of this letter of authorization, and the terms herein and any subsequent amendments that might be made to the letter of authorization, instruct them about the means through which they are to obtain and administer the vaccine under the EUA, and ensure that the authorized labeling [i.e., Fact Sheet for Healthcare Providers Administering Vaccine (Vaccination Providers) and Fact Sheet for Recipients and Caregivers] is made available to vaccination providers through appropriate means (e.g., e-mail, website). (*Id*).

The "Organizations" recruited by the state to act under color of law in the CDC COVID-19 Vaccination Program became known as "vaccination providers" under each EUA (3-ER-542, FN 6). Their duties under each EUA included, but were not limited to:

(1) "Vaccination providers will administer the vaccine in accordance with the authorization and will participate and comply with the terms and training required by CDC's COVID-19 Vaccination Program," (3-ER-546-547)

(2) monitor and report adverse reactions to VAERS, and (*Id.*)

(3) comply with all state and CDC requirements, *(Id.*)

All Appellees were required by the Provider Agreement to ensure that all potential recipients of the COVID-19 drugs were made aware that they were being offered EUA/PREP Act drugs that had "not been approved or licensed by the FDA," of the drugs' risks/benefits/alternatives, and of their right to accept or refuse the drugs without pressure or penalty. (3-ER-547) Instead, Appellees concealed Appellants' right to refuse. (3-ER-368, 455, 468)

Moreover, each EUA required Appellees to ensure that "all descriptive printed matter" "relating to the use of the Pfizer-BioNTech COVID-19 Vaccine clearly and conspicuously shall state that: this product has not been approved or licensed by the FDA, but has been authorized for emergency use by the FDA, under an EUA…" Therefore, under any "vaccination" policy involving the EUA drugs, Appellees were under a duty to inform the potential users that the drugs were not *licensed* by the FDA but rather were *authorized* under EUA, a statement Appellees failed to disclose in any of their "vaccination" policies.

G. Federal Wide Assurance Program

The federal government established the Federal Wide Assurance Program (3-ER-453) to ensure its Executive Branch never engaged in the human rights atrocities exposed by Senator Kennedy after the nation became aware of the Tuskegee Experiment, leading to revelations of thousands of other related abuses. (3-ER-348)The federal government achieved this goal by requiring any person acting under

the federal government's authority or using its funding when subjecting humans to investigational products or processes to comply with 45 CFR Part 46 and the Belmont Report, which both were established under the 1974 National Research Act. (3-ER-349, FN9) The State's FWA requires it to adhere to the regulatory scheme under 45 C.F.R. § 46.116 and the Belmont Report anytime it subjects individuals to publicly funded COVID-19 investigational products without exception as a condition of the state and its recruited parties accessing the drugs and billing for their administration. The primary ministerial duty required of the state and its recruited state actors is to obtain an individual's "legally effective informed consent," meaning the person offering the product must ensure that at no time is an individual under "coercion," "undue influence," "unjustifiable pressures" or a sanction to participate. The State's FWA is a duty placed upon the state explicitly for the Appellants' benefit to ensure the federal government is not involved in medical abuses, and it is for this reason that the CDC COVID-19 Vaccination Program was exclusively offered only through a state's existing immunization program (3-ER-497). Appellees claim the CDC COVID-19 Vaccination Program did not involve research protocols, nor were persons who submitted to a drug under the program considered human subjects. These claims are incorrect, as research is any activity that adds to the generalizable knowledge of the product. (3-ER-445, FN11) A human subject is a human whose private, identifiable information is known, and data

collected regarding their involvement with the product is monitored and studied. (45 CFR 46.102(e)(1)) The administration of an investigational drug to one person activates 45 CFR Part 46, and the institution's Institutional Review Board, as the regulatory scheme is not limited to clinical trials but broadly defines the activity to protect all persons involved in federally funded drugs. (3-ER-349) For purposes of a motion to dismiss, the court is required to accept Appellants' allegations that the CDC COVID-19 Vaccination Program required the HHS Secretary to conduct research activities[9] that he established in each EUA[10] that Appellees agreed to perform. (3-ER-463-464) All persons who submitted to an EUA injection are currently being monitored, and their data will be studied for years by medical researchers involving the drug's manufacturers and U.S. Government agencies.

Therefore, at all times material, Governor Inslee, agreeing to perform for the federal government under the state's FWA and CDC COVID-19 Vaccination Program, only had a ministerial duty to accept Appellants' freely given consent when subjecting them to publicly funded investigational new drugs, a fact Gov. Inslee does not dispute, only that one of the drugs under his mandate, "Pfizer-BioNTech COVID-19 Vaccine," was not investigational, which is directly contradicted by the FDA documents submitted by Appellants.

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[9] 21 U.S.C. § 360bbb-3(e)(1)(A)(iii)
[10] (3-ER-545, F, G) (3-ER-546, N, P) (3-ER-547, T,U,V)

Governor Inslee established a "vaccination" requirement, before the FDA licensed any COVID-19 drug with a legal indication to inoculate a person from the Coronavirus. Gov. Inslee's "vaccination" requirement mandated that Appellants inject one of the publicly funded investigational drugs into their bodies as a condition to start or continue working in the state's healthcare industry. This indisputable fact demonstrates his reckless and wanton disregard for the duties of his office and of Appellants' rights, safety, and financial security.

H. Qualified Immunity

The district court erred by granting Governor Inslee qualified immunity because it held that the drugs were not FDA-labeled as investigational as previously discussed. Appellants effectively demonstrated that Governor Inslee failed to perform ministerial duties under the state's FWA, CDC COVID-19 Vaccination Program, 21 U.S.C. § 360bbb-3, and the PREP Act, that his Proclamation, as applied, was outside the scope of his authority to issue, and that his willful and reckless violation of his duties directly led to the deprivation of Appellants' Fourteenth Amendment rights which facts deny him qualified immunity.[11] (3-ER-371-375). The FDA clearly stated that "In an emergency, it is critical that the

[11] While Gov. Inslee may argue that he was not the one administering the shots and interacting with the recipients, his Proclamation demanded that any healthcare worker who exercised the right to refuse would be penalized and such requirement was outside the scope of his authority to mandate.

conditions that are part of the EUA--those that the Commissioner has determined to be necessary or appropriate to protect the public health--be strictly followed, and that no additional conditions be imposed. (3-ER-404).

This Court has held that government officials are "unprotected" from qualified immunity when they refuse to perform a ministerial duty.[12] Courts have long held that "Qualified immunity protects "all but...those who knowingly violate the law *Mullenix v. Luna*, 577 U.S. 7, 136 S. Ct. 305, 193 L. Ed. 2d 255 (2015) and "Qualified immunity is only available when an official acts 'within the scope of [his or her] discretionary authority.'' *Brooks v. George County,* 84 F.3d 157, 164-65 (5th Cir. 1996) And the Tenth Circuit holds that qualified immunity is not a defense for an official that "failed to actually comply with the statute upon which they purportedly relied" (*Roska v. Sneddon*, 437 F.3d 964 (10th Cir. 2006)). Governor Inslee claims that he was not subject to the EUA Statute, but did not dispute Appellants' allegations regarding his duties under the PREP Act statute and the state's FWA.

I. The district court erred by granting PeaceHealth's Motion to Dismiss

Appellants alleged that PeaceHealth volunteered to perform for the State of Washington under the CDC COVID-19 Vaccination Program. Moreover,

[12] *Groten v. California*, 251 F.3d 844 (9th Cir. 2001)

Appellants alleged that PeaceHealth's vaccination requirement exclusively relied on drugs only authorized for emergency use under the EUA Statute, offered exclusively through the CDC COVID-19 Vaccination Program, immunized from liability under the PREP Act, and FDA-labeled as investigational involving PeaceHealth's duties under its FWA. Moreover, Appellants alleged that since those facts are true, PeaceHealth could not subject them, under threat of penalty, to the drug's use, and such requirements were preempted by the Supremacy Clause as it relates to the EUA Statute and express preemption language under the PREP Act. Appellants also alleged that PeaceHealth was a state actor with a legal obligation to perform the ministerial function of accepting Appellants' chosen option under the Provider Agreement on behalf of the state of which their vaccination policy violated their fundamental Fourteenth Amendment rights to Equal Protection, Due Process, and Privacy and also their statutory rights to refuse products and activities under the EUA Statute and PREP Act without penalty or pressure. (3-ER-Dkt 8)

PeaceHealth's defense is built upon the fallacy that the drugs were not investigational but actually FDA-approved, conditions which means they were not subject to the EUA Statute, the Provider Agreement, or their FWA.

The district court accepted PeaceHealth's claim that the drugs were not investigational and issued a ruling on each count as if the question before the court involved a vaccine mandate when, in fact, it involved an EUA/PREP Act

investigational drug mandate. Therefore, the ruling is reversible error as it relates to each count. Moreover, the district court only cited to cases addressing true vaccine mandates with licensed drugs, which is of no help to this Court since the mandates in question involve only unlicensed EUA/PREP Act investigational drugs.

J. State Action

The federal government owed constitutional obligations to all persons in Washington state, which Governor Inslee agreed to fulfill on behalf of the federal government and which PeaceHealth agreed to perform on behalf of the state. By signing the Provider Agreement, PeaceHealth became part of a ***collaboration*** with the federal and state governments. The drugs were federally funded, they were required to be distributed and administered through the state's existing immunization cooperative agreement with the federal government, and very specific data was to be collected by PeaceHealth and reported through the state's system or through the federal VAERS database. Significantly, PeaceHealth was prohibited from charging recipients for the shots or their administration, and PeaceHealth was immunized from liability under the PREP Act (3-ER-498) in return for its lawful participation in the program. All of this was done in order to achieve the federal and state governments' public function of distributing and administering EUA/PREP Act investigational drugs during a national medical emergency. A key indicator that PeaceHealth was acting under color of law is to ask whether it could have performed

any of those activities on its own. The answer is no. PeaceHealth could not conduct itself as it does when acting as a private party healthcare provider. PeaceHealth could not have purchased the drugs directly from Pfizer, administered them to patients, and charged for their services. That private party activity was strictly forbidden under the Provider Agreement. Thus, whenever PeaceHealth interacted with any person, at any time, regarding the CDC Vaccination Program COVID-19 drugs, it was acting under color of law, whether those persons were patients or employees.

The district court did not accept Appellants' state action arguments specifically because it held that the drugs were not investigational, under EUA, and therefore, not subject to the terms of the CDC COVID-19 Vaccination Program relating to the use of the federal government's investigational drugs. However, Appellants provided the court with ample facts in their complaint (3-ER-461-468) and opposition to Appellees MTD (3-ER-227-250), demonstrating that at all times material, PeaceHealth was acting under color of law when involving Appellants with the federally funded program.

The district court dismissed PeaceHealth for the same reasons it dismissed Gov. Inslee. Therefore, Appellants' arguments for why it was reversible error to dismiss Gov. Inslee apply equally to the district court's dismissal of PeaceHealth, and they are incorporated as if included *in extenso*.

K. Appellants' Counts

The District Court erred by ruling on each count as if the argument before it only involved licensed drugs and case law pertaining to true vaccine mandates. Appellants were unambiguous that they "make no assertions regarding whether it is lawful for a public or private entity to mandate taking a ***licensed*** vaccine. Plaintiffs' allegations herein only relate to Defendants' mandating the use of drugs, biologics, and devices under 21 U.S.C. § 360bbb-3 and PREP Act protocols." (3-ER-439)

Therefore, because the district court ruled on each count under the false premise that the drugs were not investigational, not under EUA, and not subject to the Provider Agreement and applicable law, Appellants wish to demonstrate to this Court why the lower court's ruling is reversible error.

i. Count One "Subjected to Investigational Drug Use" (3-ER-481)

Governor Inslee's Proclamation, relying exclusively on the use of publicly funded investigational drugs, deprived Appellants of their constitutional, statutory, and contractual third-party beneficiary right to refuse the federal investigational drugs without penalty or pressure as conferred upon them by Congress in the above-cited statutes and their fundamental right to refuse unwanted investigational medical treatment under the Fourteenth Amendment. PeaceHealth, acting under color of law when subjecting Appellants to the federal investigational drugs, its "vaccination" policy, and acting upon Governor Inslee's unlawful Proclamation deprived

Appellants of their unqualified right to refuse the investigational drugs without consequence. Moreover, at all times material, Appellees were completely preempted by federal law from establishing a "vaccination" requirement relying on the federal investigational drugs and enforcing penalties upon Appellants when they exercised their right to refuse. Appellants had the express right to refuse the federal investigational drugs mandated by Appellees pursuant to the EUA Statute, each EUA, the COVID-19 Vaccination Program and Provider Agreement, 10 U.S.C. § 980, and the Fourteenth Amendment's prohibition on requiring an individual to involuntary surrender their right to privacy, procedural and substantive due process, and equal protection. These rights implicate fundamental rights which are enforceable pursuant to 42 U.S.C. § 1983 because at all times material Appellees were acting under color of law.

ii. Count Two "Unconstitutional Conditions Doctrine" (3-ER-

The Unconstitutional Conditions Doctrine reflects the U.S. Supreme Court's repeated pronouncements that the government "may not deny a benefit to a person on a basis that infringes his constitutionally protected interests,"[13] nor can a person "barter away his life or his freedom, or his substantial rights" (*Home Ins. Co. of New York v. Morse*, 87 U.S. 455, 451 (1874)), and "…[t]he state, having power to deny a

[13] *Koontz v. St. Johns River Water Mgmt. Dist.*, 570 U.S. 595, 133 S. Ct. 2586, 186 L. Ed. 2d 697, 24 Fla. L. Weekly Fed. S 435 (2013)

privilege altogether, may grant it upon such conditions as it sees fit to impose. But the power of the state in that respect is not unlimited; and one of the limitations is that it may not impose conditions which require the relinquishment of constitutional rights. If the state may compel the surrender of one constitutional right as a condition of its favor, it may, in like manner, compel a surrender of all. It is inconceivable that guaranties embedded in the Constitution of the United States may thus be manipulated out of existence" *Frost Trucking Co. v. R.R. Com*, 271 U.S. 583, 593-94 (1926). Governor Inslee's Proclamation resulted in an unconstitutional condition because he utilized the EUA Statute and PREP Act "to 'produce a result which (it) could not command directly'"[14] and "[s]uch interference with constitutional rights is impermissible."[15]

Governor Inslee's Proclamation established an unconstitutional requirement that Appellants must surrender their Fourteenth Amendment due process rights to bring common law causes of action for product liability, medical malpractice, fraud, and battery; their rights to seek tort remedies for bodily harm caused by another member of society; and their rights to be made whole for damages to their finances and emotional well-being arising out of a covered countermeasure under the PREP Act as a condition of working anywhere within the state's healthcare industry.

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[14] *Perry v. Sindermann*, 408 U.S. 593 (1972), citing *Speiser v. Randall*, 357 U.S. 513, 526, 78 S.Ct. 1332, 1342, 2 L.Ed.2d 1460 (1958).
[15] *Id.*

Governor Inslee's Proclamation established an unconstitutional condition because he required Appellants to surrender their Fourteenth Amendment rights to privacy involving investigational drugs and their rights to a procedural due process before losing their statutory entitlement to refuse as a condition of working in the state's healthcare industry. Governor Inslee's Proclamation established an unconstitutional condition on the grounds that it required Appellants to inject unwanted investigational drugs into their bodies as a condition of working in the state's healthcare industry.

PeaceHealth, acting under color of law at all times material, established a "vaccination" policy as an "Organization" under the Provider Agreement and "Vaccination Provider" under each EUA, and as a medical facility under Governor Inslee's Proclamation that established an unconstitutional condition by requiring Appellants to use investigational products only authorized under the PREP Act, EUA Statute, and Provider Agreement as previously discussed as a condition of starting or continuing employment. PeaceHealth, acting under color of law, was preempted by previously discussed laws, regulations, and agreements from establishing those conditions, depriving Appellants of their constitutional protections.

iii. Count Three "Equal Protection" (3-ER-482)

Governor Inslee's Proclamation denied Appellants their equal protection under the EUA Statute, the PREP Act, and the State's FWA by only penalizing the option to refuse because that option interfered with his personal goals, which were outside the scope of his authority to enforce. The Supreme Court "explained that '[t]he purpose of the equal protection clause of the Fourteenth Amendment is to secure every person within the State's jurisdiction against intentional and arbitrary discrimination, whether occasioned by express terms of a statute or by its improper execution through duly constituted agents' *Sioux City Bridge Co., supra,* at 445 (quoting *Sunday Lake Iron Co.* v. *Township of Wakefield,* 247 U. S. 350, 352 (1918))." *Village of Willowbrook v. Olech*, 528 U.S. 562 (2000)

PeaceHealth, acting under color of law, discriminated against Appellants for the sole reason that they disagreed with Appellants' fundamental, statutory, and third-party beneficiary chosen option.

Appellants were those healthcare workers, similarly situated with all state healthcare workers, choosing the option to refuse and therefore the claims against Appellees are enforceable pursuant to 42 U.S.C. § 1983.

iv. Count Four "Due Process" (3-ER-483)

Governor Inslee's Proclamation, relying exclusively on the use of investigational drugs, violated Appellants' fundamental right to refuse unwanted

investigational drugs. He deprived Appellants of their substantive due process rights to use their statutory entitlements to refuse under the PREP Act, EUA Statute, the state's FWA, and the CDC COVID-19 Vaccination Program (a federal program). He completely deprived Appellants of their fundamental right to work in the state's healthcare industry when they refused the investigational drugs. He deprived Appellants of their substantive due process rights to refuse the surrendering of their right to sue if injured by an EUA/PREP Act drug by penalizing the option to refuse. (3-ER-367). Moreover, Governor Inslee never established a date, time, or place for Appellants to air their complaint before an impartial body depriving them of their property interest.

The Supreme Court established a two-pronged approach[16] to analyze a substantive-due-process claim. First, analyze whether the claimed fundamental rights and liberties is deeply rooted in this nation's history and tradition. *Moore v. East Cleveland*, 431 U. S. 494 (1977). Second, set forth a "careful description" of the asserted fundamental liberty interest. *Reno v. Flores*, 507 U. S. 292 (1993)

Appellants' "careful description" of their asserted fundamental liberty interest is that they have a fundamental right to refuse an investigational drug without penalty or pressure. Regarding being deeply rooted, starting in 1938 (the FDC Act), the federal government prohibited drugs from being introduced into commerce before

[16] *Washington v. Glucksberg*, 521 U.S. 702 (1997)

the FDA approved them for general marketing according to their labeling (21 U.S.C. § 355(a)). In 1974, Congress passed the National Research Act requiring an institutional review board to monitor research activities involving humans and unapproved therapeutics and devices. In 1978, the Belmont Report was published and formed the basis of the Common Rule (45 C.F.R. Part 46). In 1992, the U.S. Senate ratified Article VII of the ICCPR Treaty. In 2001, the federal government required all persons conducting business with its agencies, departments, or the military to have an FWA on file before they use federal funding when involving humans with investigational medical products. The State of Washington has over 500 active FWAs out of 30,000 nationwide.

Moreover, The entire federal government's budget must filter through 45 C.F.R. § 46.122 (Use of Federal Funds), and DoD funding must filter through 10 U.S.C. § 980 (Limitation on use of humans as experimental subjects) when the funding involves humans with investigational drugs. Congress places strict requirements on involving humans with investigational drugs under 21 U.S.C. 321, 331, 351, 352, 355, 360bbb *et. seq*., and 371; 42 U.S.C. 262; 10 U.S.C. § 1107; 45 C.F.R. Part 46; 21 C.F.R. §§ 50,56; 21 C.F.R. Part 312; and the Belmont Report. The primary requirement of the statutes and agreements is that a person offering a federally funded investigational product must ensure that the potential recipient is never under outside pressure to participate, such as "sanctions," "coercion," "undue

influence," and "unjustifiable pressures." The right to refuse an investigational drug is a fundamental right that is pervasive, historical, and deeply rooted in the $600B pharmaceutical research industry and this nation. No constitution, statute, or regulation, provides any person authority to require another person to be injected with an investigational drug under threat of penalty or by pressure and the Supremacy Clause dictates that all authorities are preempted from requiring that which Congress expressly prohibits—nonconsensual use. An investigational drug does not have a legal indication to treat, cure, or prevent any known disease or virus, nor is it licensed by the FDA for its safety, thus, the drug, at best, can only be considered investigational medical treatment, which Appellants have the fundamental right to refuse as noted in *Health Freedom v. Carvalho*, 22-55908 (9th Cir June 7, 2024) citing *Cruzan v. Director, Mo. Dept. of Health*, 497 U.S. 261, 278-79 (1990); *Washington v. Glucksberg*, 521 U.S. 702 (1997) which Appellees, acting under color of law, violated.

Therefore, Appellees violated Appellants' substantive due process rights to refuse unwanted investigational drugs and their procedural due process rights by depriving them of their statutory entitlements without a hearing before an impartial body and those violations are enforceable pursuant to 42 U.S.C. § 1983.

v. Count Five "Spending Clause Doctrine" (3-ER-484)

The Supreme Court held in *Health and Hospital Corporation of Marion County v. Talevski*, 599 U.S. 166 (2023) that a federal statute having rights creating language for an individual is presumed enforceable under § 1983 when the statute does not contain a specific civil right of action. Specifically the court stated:

> *Gonzaga* sets forth our established method for ascertaining unambiguous conferral. Courts must employ traditional tools of statutory construction to assess whether Congress has "unambiguously conferred" "individual rights upon a class of beneficiaries" to which the plaintiff belongs. [citations omitted] Notably, it must be determined that "Congress intended to create a federal right" *for* the identified class, not merely that the plaintiffs fall "within the general zone of interest that the statute is intended to protect" *Gonzaga*, 536 U.S., at 283. This paradigm respects Congress's primacy in this arena and thus vindicates the separation of powers. *Id*., at 286. Id., at 284, 287. Conversely, we have rejected § 1983 enforceability where the statutory provision "contain[ed] no rights-creating language"; had "an aggregate, not individual, focus"; and serve[d] primarily to direct the [Federal Government's] distribution of public funds." *Id*., at 290
>
> We have held that the *Gonzaga* test is satisfied where the provision in question is "phrased in terms of the persons benefited" and contains "rights-creating," individual-centric language with an "unmistakable focus on the benefited class."

The Court held that the "right to be free from . . . any physical or chemical restraints" and the right to advanced notice of discharge provisions of the FNHRA statute "meet[s] this test," stating that "This framing is indicative of an individual "rights-creating" focus. *Gonzaga*, 536 U. S., at 284."

While the federal statutes involved in the instant action and *Talevski* are different, the legal analysis is identical. Appellants claim that Congress created a right for them under 21 U.S.C. 360bbb-3(e)(1)(A)(ii)(III), in pertinent part:

> The Secretary...shall establish...conditions...to ensure that individuals to whom the product is administered are informed...of the option to accept or refuse administration of the product...

This language "unambiguously confers" the right to be informed of the option to accept or refuse administration of the product. Also, it speaks in terms of "individual rights upon a class of beneficiaries" to which Appellants belong. The provision actually uses the word "individuals" when describing to whom the right is conferred. The class of beneficiaries are those contemplating the "administration of the product." By describing the right the way it did, Congress intended to create a federal right for Appellants.

The PREP Act expressly preempts any person from interfering in any requirement applicable to a countermeasure under the EUA Statute. Thus, Congress incorporated any requirement under FDCA into the PREP Act, which includes the rights-creating language under 21 U.S.C. § 360bbb-3(e)(1)(A)(ii)(III), and such language creates a § 1983 cause of action.

Talevski specifically extended § 1983 to federal contracts involving an individual's statutory entitlement, and thus the Provider Agreement requires

Appellees to comply with the FDCA, any EUA, and all applicable state vaccination laws, which incorporate the rights creating language under 21 U.S.C. § 360bbb-3(e)(1)(A)(ii)(III). The CDC Program was designed to administer federal investigational drugs to individuals but under the explicit condition that all persons had the explicit right to accept or refuse without consequence or being placed under "sanctions," "coercion," "undue influence," or "unjustifiable pressures."

Appellees, acting under color of law, violated Appellants' rights to refuse. Moreover, because 10 U.S.C. § 980 places a duty upon any person offering another person DoD-funded investigational drugs to prospectively obtain their informed consent, the duty creates a right of the potential recipient to give informed consent, which must be "legally effective." Therefore, Appellees subjected Appellants to DoD funded drugs and did not obtain their legally effective informed consent in violation of Appellants' 10 U.S.C. § 980 rights and enforceable pursuant to 42 U.S.C. § 1983.

vi. Count Six "Breach of Contract" (3-ER-487)

The district court erred by holding that the drugs under Appellees "vaccination" requirements were not investigational drugs subject to the Provider Agreement. The Provider Agreement required Appellees to perform the ministerial duty of accepting an individual's freely chosen option under the EUA Statute and all EAUs. Moreover, the Provider Agreement required Appellees to provide Appellants

with a drug Fact Sheet and inform them of the products' risks/benefits/alternatives, that the drugs were not licensed by the FDA, and of their right to accept or refuse. Neither the Provider Agreement, nor the EUA Statute, nor any EUA provided Appellees authority to establish "vaccination" requirements requiring any individual to involuntary be injected, which demonstrates that Appellees amended the Provider Agreement by amending each EUA's "condition of authorization" to exempt themselves from performing their ministerial duties.

Appellees breached Appellants' third party beneficiary rights when requiring what the Provider Agreement explicitly prohibits—nonconsensual use. Appellees did not dispute that they had a duty to perform for Appellants under the Provider Agreement; they only dispute that the drugs were not investigational and thus implied that the Agreement's duties did not come into being.

vii. Count Seven "Employment Tort" (3-ER-488)

The district court erred by holding the drugs were not investigational and that Appellants did not have the right to refuse. Appellants alleged that "Defendants unlawfully terminated Plaintiffs' employment when Plaintiffs exercised a legal right or privilege" (3-ER-489, ¶ 249). Washington State common law (3-ER-489, FN65) prohibits Governor Inslee requiring medical facilities to terminate Appellants' employment for exercising their right to refuse investigational drugs. The same common law prohibits PeaceHealth in the same manner.

viii. Count Eight "Outrage" (3-ER-490)

Appellants have alleged sufficient facts to establish a claim of outrage, demonstrating that Appellees knew they could not destroy the careers of healthcare professionals refusing investigational drugs. The district court erred by not accepting Appellants' allegations as true and dismissing this claim.

ix. Count Nine "Invasion of Privacy"(3-ER-491)

Appellants alleged enough facts to demonstrate that PeaceHealth engaged in conduct invading Appellants' rights to privacy, requiring them to publicly disclose their private health information, painting them in a false light publicly, which led to malicious harassment by fellow staff members and the public. Moreover, PeaceHealth knowingly committed malicious acts to intentionally cause Appellants emotional distress to coerce them into surrendering their constitutional and statutory rights.

The district court erred by holding the drugs were not classified as investigational. Appellees could not require Appellants to disclose to them when and if they used an investigational drug nor require them to involuntarily disclose their private identifiable and health information to unknown persons for unknown reasons as a condition of working in the state's healthcare industry or within PeaceHealth. PeaceHealth also did not have the authority to require Appellants to wear a badge publicly disclosing their private health information, but did so anyway.

x. Count Ten "Implied Private Right of Action"

The district court erred by not judicially creating an implied private right to action to refuse under 21 U.S.C. § 360bbb-3(e)(1)(A)(ii)(III). Appellants alleged that Congress implied a private right of action under 21 U.S.C. § 360bbb-3(e)(1)(A)(ii)(III) when creating a right for their benefit that is deeply rooted in American culture, history, and fundamental bodily integrity rights involving investigational medical products. Either Appellants have a statutory entitlement to refuse an EUA drug or they do not. If the option to refuse is a property interest held by Appellants, then it must be Appellants' right to enforce via a private right of action that nearly sixty years of legislative action by the U.S. Congress supports.

The district court erred by dismissing Appellants' claims with prejudice. Although Plaintiffs did file an Amended Complaint, that amendment was a requirement of the court for Appellants to remove all references to the legislation and history of the novel laws rarely discussed in federal district courts from the Complaint. Therefore, even though the Ninth Circuit holds that "[W]hen a district court has already granted a plaintiff leave to amend, its discretion in deciding subsequent motions to amend is 'particularly broad,'" *Chodos v. W. Publ'g Co.*, 292 F.3d 992, 1003 (9th Cir. 2002) is not applicable to the instant action because Appellants were not afforded an opportunity to amend their Complaint based on the own accord.

L. *Biden v Missouri*

The district court erred, citing *Biden v. Missouri*, 595 U.S. 87, 91 (2022), which only related to vaccine mandate authority, not the authority to mandate the use of investigational drugs. (1-ER-31) The district court stated that "failure to comply may lead to monetary penalties" relating to the Medicare rule. However, failure to comply with the Provider Agreement could lead to criminal penalties involving laws different from those under the Medicare rule and thus not applicable to the instant action.

The district court erred by misapplying the EUA Statute when stating that the statute only applies to the physical administration of the product. (1-ER-38) First, the EUA Statute requires the HHS Secretary to establish conditions ensuring the individual is informed of their right to accept or refuse, which, under each EUA, was assigned first to the State of Washington as the "Emergency Response Stakeholder" and then the state, having identified the Organization (PeaceHealth) to administer the drug on its behalf, assigned that duty at the executive level known as the "vaccination provider." (2-ER-Dkt. 36-1) Therefore, the Organization (PeaceHealth) is required to ensure all persons are informed of the right to refuse and cannot establish a policy conflicting with that requirement. Second, "administration" requires the legal function of determining whether the potential recipient is willing to give informed consent. Governor Inslee and PeaceHealth offered Appellants the

investigational drugs, and Appellants exercised the right to refuse. It would be an absurd result to determine whether someone wanted to exercise their right to refuse only after the investigational drug is injected into their body. Lastly, and most importantly, the district court erred by not understanding that the right to refuse is inherent. A person is not required to go to a healthcare professional offering the drug to be able to refuse; they can simply refuse to go to any healthcare professional offering the drug to begin with.

The right is a statutory entitlement owed to all persons. No matter the levels of delegation from the federal government to PeaceHealth, that right was not abrogated, and neither Gov. Inslee nor the State nor PeaceHealth can demonstrate a basis for their alleged authority to define a category of individuals who cannot exercise that right. The federal government did not afford Appellees an opportunity to amend the conditions under which they volunteered to perform and to require of anyone that which Congress expressly prohibits, nonconsensual use of the federal investigational drugs, nor have they offered such authority for the court to review. The nonsensical claim that simply because Appellees did not require Appellants to receive the shot from them (1-ER-38) somehow authorizes them to require the shot as a condition of working anywhere in the state's healthcare industry, or specifically at PeaceHealth, under the state's licensing requirements is a reckless expansion of the EUA Statute and the PREP Act. At all times material, the duties owed under the

federal programs, laws, and regulations were owed to "individuals" without exception to who those individuals were. Can PeaceHealth create a company policy requiring volunteers, contractors, visitors, patients, to participate in investigational drugs outside of their freely given consent simply because PeaceHealth created a policy? PeaceHealth's Policy cannot reconcile with the rights of individuals under PeaceHealth's Institutional Review Board (45 CFR Part 46), FWA, EUA Statute, and its federally funded programs. The district court erred by creating a judicial procedural device allowing Appellees to circumvent the well-established intent of Congress involving investigational drugs authorized for use only under a declared emergency. The entire federal government must ensure that not a single dollar of its budget is used to coerce a person into involuntary use of an investigational drug. The district court erred by not protecting Appellants' rights.

CONCLUSION

For the foregoing reasons, the judgment of the district court should be reversed and rendered (1) invalidating Gov. Inslee's and PeaceHealth's investigational drug mandate, and (2) holding that PeaceHealth was operating under color of law when penalizing anyone for refusing an EUA/PREP Act investigational drug, and (3) remanded for a ruling on all issues not addressed by the trial court.

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STATEMENT REGARDING ORAL ARGUMENT

As required by 9th CIR. R. 32(f) and FED. R. APP. P. 34(a)(1), Appellants stand ready for oral argument should the court believe it is necessary.

UNITED STATES COURT OF APPEALS
FOR THE NINTH CIRCUIT

Form 17. Statement of Related Cases Pursuant to Circuit Rule 28-2.6

9th Cir. Case Number(s) _____24-1869_____

The undersigned attorney or self-represented party states the following:

[ ] I am unaware of any related cases currently pending in this court.

[ ] I am unaware of any related cases currently pending in this court other than the case(s) identified in the initial brief(s) filed by the other party or parties.

[X] I am aware of one or more related cases currently pending in this court. The case number and name of each related case and its relationship to this case are: 24-1949

Signature *s/ David J. Schexnaydre* **Date July 15, 2024**
(use "s/[typed name]*" to sign electronically filed documents)*

UNITED STATES COURT OF APPEALS
FOR THE NINTH CIRCUIT

Form 8. Certificate of Compliance for Briefs

9th Cir. Case Number(s) _____24-1869_____

I am the attorney or self-represented party.

This brief contains 13,658 words, excluding the items exempted by Fed. R. App. P. 32(f). The brief's type size and typeface comply with Fed. R. App. P. 32(a)(5) and (6).

I certify that this brief *(select only one)*:

[X] complies with the word limit of Cir. R. 32-1.

[ ] is a **cross-appeal** brief and complies with the word limit of Cir. R. 28.1-1.

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[ ] is for a **death penalty** case and complies with the word limit of Cir. R. 32-4.

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- [ ] it is a joint brief submitted by separately represented parties;
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[ ] is accompanied by a motion to file a longer brief pursuant to Cir. R. 32-2(a).

Signature *s/ David J. Schexnaydre* **Date: July 15, 2024**
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